

Andrew Blumenfeld MD, Neurology, Coastal Medical Group

Date: 11/29/2023

PATIENT NAME: Steve Ronald Handrop (stevehandrop@yahoo.com)

DATE OF BIRTH: 12/7/1954

DATE OF ACCIDENT: 10/18/2023

NEUROLOGY FOLLOW-UP

CURRENT COMPLAINTS:

This is a tele-visit.

He consents to this.

His headaches have resolved.

His tinnitus is starting to improve and is now only present about 50% of the time.

Today he has no tinnitus.

He has no nausea or vomiting.

He has ongoing neck and back pain, rated at 5-7 out of 10.

He is scheduled for a cervical epidural treatment.

He notes intermittent arm discomfort, but has no persistent numbness or weakness in his arms or legs.

He still has mild difficulty with concentration and short-term memory problems.

MEDICAL HISTORY:

Past Medical History:

He has no history of hypertension or TIA.

He is pre-diabetic. HBA1C in the 6 range. He is followed by his PCP for this and is on active treatment.

Sleep apnea on CPAP.

SOCIAL HISTORY:

He is married.

He is a pilot and is anxious to get back to flying.

He is a non-smoker.

His caffeine intake is minimal.

His alcohol use is zero.

NEUROLOGICAL PHYSICAL EXAMINATION

CERVICAL REGION:

Patient:

Date:

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Limited neck movements in all directions.

MENTAL STATUS EXAM:

Alert with normal attention.

He has no aphasia or dysarthria.

Affect is consistent with irritability.

Orientated to place, person, and time.

Cognitive function shows adequate recent and remote recall as well as attention span.

CRANIAL NERVES:

There is no diplopia.

Hearing is poor.

DIAGNOSTIC IMAGING:

Brain MRI scan 11/20/23: multiple patchy and confluent areas of white matter disease. No hemorrhages or areas of encephalomalacia seen.

NEUROLOGICAL IMPRESSION:

Concussion without loss of consciousness

Post traumatic syndrome with whiplash injury with cervical myofascial pain and disc changes.

The white matter changes seen on his brain MRI scan could be to trauma or related to small vessel disease. His main risk factor for this is pre-diabetes, but the changes noted are more than I would expect with a pre-diabetic state alone.

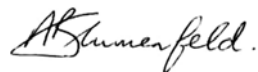
RECOMMENDATIONS:

- CPAP ongoing
- Orthopedic follow up for ongoing spine issues.
- Aspirin 81 mg qd

FOLLOW UP:

The patient will follow up with me if he has an increase in neurological symptoms.

Sincerely,



Andrew Blumenfeld MD, FAAN, FAHS

Diplomate, American Board of Psychiatry and Neurology

Certified in Headache Medicine

Fellow of the American Academy of neurology

Patient:

Date:

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Fellow of the American Headache Society